# CODING DICTIONARY VERSION LOG

Kelvaparib programme
Kelvaparib (KLV-3308)
Marnholt Oncology GmbH

## 1. PURPOSE

This log records the coding dictionary and version in force for each study in the kelvaparib programme at each data cut. It is the reference against which a dictionary attribution in a clinical document is checked. Where a document states a version that differs from the version recorded here for that study and data cut, the discrepancy is a defect and is resolved before the document is issued.

Two dictionaries are used in this programme. Adverse events, medical history and indications are coded to the Medical Dictionary for Regulatory Activities. Concomitant and prior medications are coded to the WHO Drug Dictionary. Every coded field is expected to carry an attribution naming its dictionary and version, in the methods section of the document and as a footnote to every table presenting coded terms.

## 2. VERSIONS IN FORCE BY STUDY

| Study | Data cut | MedDRA version | WHODrug version |
|---|---|---|---|
| KLV-7702 | 3 April 2019 | 22.0 | Global B2, March 2019 |
| KLV-7702 | Re-code, March 2022 | 25.0 | Global B3, March 2022 |
| KLV-7704 | 30 April 2020 | 23.1 | Global B2, September 2020 |
| KLV-7701 | 17 November 2021 | 24.1 | Global B3, September 2021 |
| KLV-7701 | Re-code, March 2022 | 25.0 | Global B3, March 2022 |
| KLV-7703 | 10 January 2023 | 25.0 | Global B3, March 2022 |

A re-code to a later dictionary version was performed for the studies contributing to the marketing application, so that terms are consistent across the integrated summaries. Where a study appears twice, the later row is the version in force for a clinical document issued after the re-code; a document issued before it states the earlier row (section 3).

## 3. RE-CODING AND ITS CONSEQUENCES

A change of dictionary version can change the preferred term to which a verbatim term maps, and can therefore change the count reported against a term. Where a document presents data coded to more than one version, the version applying to each data cut is stated separately and the counts are not pooled across versions.

The re-code performed in March 2022 moved every contributing study to MedDRA 25.0 and WHODrug Global B3. Documents issued before that date state the earlier version and remain correct as issued; they are not revised retrospectively.

## 4. EXPECTED ATTRIBUTION IN CLINICAL DOCUMENTS

- A clinical study report states the MedDRA version in its methods section and as a footnote to every table of adverse events, medical history or indications.
- A clinical study report states the WHODrug version in its methods section and as a footnote to every table or listing of concomitant or prior medications.
- A listing that presents a coded field carries the same attribution as the table

derived from it.

- Where a study collected concomitant medication data, the absence of any WHODrug attribution in the clinical study report is a defect, whether or not the report presents a concomitant medication table.

## 5. CUSTODY OF THIS LOG

This log is maintained by the programme data management function and is updated whenever a dictionary version changes or a re-code is performed. The edition and effective date on each page identify the state of the log at the time a document was checked against it.